rela Hospital

BREASTFEEDING HANDBOOK

"Breastfeeding isn't just about milk;
It is also about love."

About Us

Rela Hospital is a leading quaternary care facility, Dedicated to providing world-class healthcare through advanced treatments and cutting-edge technology. Under the guidance of Prof. Mohamed Rela, a globally recognized expert in liver surgery and transplantation, we focus on delivering exceptional care with compassion and expertise.

How are we different?

- 450 bedded hospital
- Multi-speciality, Quaternary care hospital
- World renowned doctors
- 12 Centres of Excellence
- State of the art infrastructure & equipment
- 130 critical care beds
- 36 acre campus
- 14 futuristic operation theatres

At Rela Hospital, we are committed to providing comprehensive, high-quality healthcare, combining Innovation, trust, and affordability to meet the needs of both local and international patients.

Human milk is the best nutrition for all babies, including premature and sick newborns
It is recommended to exclusively breastfeed (EBF) for the first 6 months
Continue breastfeeding for 2 years and beyond, along with solid foods after 6 months
If you experience any challenges with breastfeeding, seek guidance from your doctor or lactation consultant

Why Breastfeed?

- Breast milk has all the nutrients your baby needs in the right amounts
- It lowers the risk of infections, allergies, asthma, and obesity in babies
- Breastfeeding strengthens the emotional connection between you and your baby
- Breast milk is always clean, at the right temperature, and ready to use
- It helps the uterus recover after birth and may reduce the risk of certain cancers

Feeding Frequency & Duration

- Stimulate and wake your baby before feeding
- In the first month, baby needs about 8-12 feeds in 24 hours
- A feeding session averages about 20-25 minutes
- Feeding frequency and duration decrease as your baby grows
- Follow demand feeding instead of a strict schedule

Infant Cues (From Early Cues to Late)

- Searching and rooting response
- Head moves towards voice, mouth opens
- Lips smack, tongue reaches
- Feet and legs crawl, arms push against mother to 'get' to breast
- Fists clench and move to mouth
- Exaggerated movements
- Fussing behaviour
- Brow furrows
- Crying is a LATE sign of hunger

Dos

- Massage breasts before feeding
- Use reverse pressure softening for flat/inverted nipples & engorged breasts
- Feed with support & backrest
- Mother to drink something before feeding
- Ensure latch is proper and pain free

- **Baby's mouth should be wide open**
- **Chin should touch the breast**
- **Baby's lip should be turned outward (flanged) around the areola**
- **Baby's entire body should be turned towards you**

- Sore nipples: Use expressed breast milk or nipple cream recommended by your doctor
- Ensure baby empties at least one breast to get hind milk
- Skin-to-skin contact as much as possible
- Observe baby's urine and bowel output carefully
- Drink at least 3-4 litres of fluids
- Eat a healthy diet (No food restrictions unless explicitly mentioned)
- Try to squeeze in as much rest as possible

Don'ts

- Don't leave baby on the breast too long (risk of exhaustion or weight loss)
- Don't assume every cry is for feeding. Sometimes babies just need soothing
- Don't lean forward while feeding
- Don't introduce bottes or pacifiers
- Don't give water or homemade medications
- Don't feed while breast is engorged. Don't use a breast pump on an engorged breast. Hand express a little to soften the breast. Then feed the baby

Weight of Newborns

The average newborn weighs between 2.5kg to 3.5kg. They lose 5% to 10% of birth weight following birth and then stabilize within a few days. Generally speaking, infants double their birth weight by about 5 months and triple their weight about 1 year of age and quadruple by 2 years of age. At one month, babies gain about 30g per day and about 19g per day at 4 months.

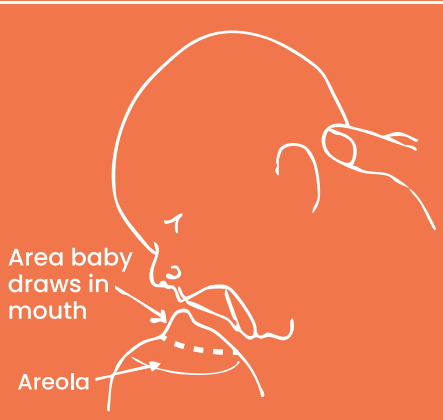
Common Breastfeeding Positions

Position	Advantages
Cradle	• It's usually the easiest and most comfortable position for you once your baby is nursing well • Many mothers use it after learning to breastfeed, because it's convenient for a bigger baby
Cross Cradle	• Supporting your baby's neck and shoulders with your hand instead of your forear allows you and the baby to have more control of his head • It's usually the easiest position to learn how to latch him onto your breast • It's especially helpful for a premature baby or a baby who's having difficulty latching onto your breast
Clutch/Football	• It's easy to see that your baby has latched on effectively • It may be the most comfortable if you had a caesarean, because your baby isn't pressing on your incision • It is helpful if you have large breasts, because your baby's chest helps support your breast

** If you are unable to establish a successful feed in the above positions, talk to your Lactation Consultant about exploring other positions that can be personalized for you.

Signs of Correct Latch

Mother's View While Latching Baby

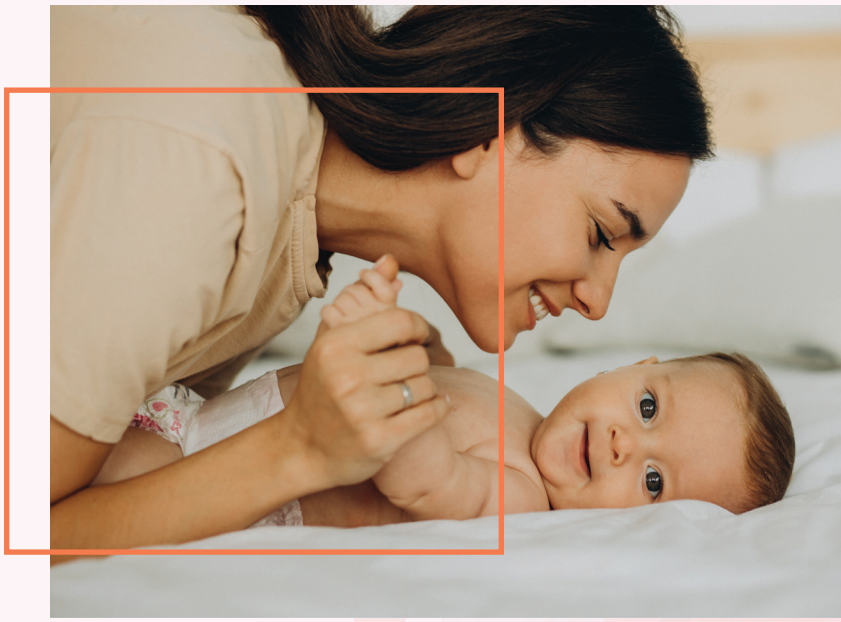


- Baby's head tilted slightly back
- Bring baby in quickly
- Gently push with base of hand on shoulders
- Only chin and cheeks touch breast
- Baby's body close against mother

Move baby, not breast

Signs of a Correct Asymmetrical Latch

- No pain (remember, tenderness is common in the first 14 days postpartum due to progesterone levels)
- More of areola is taken in with lower lip than upper lip
- Chin touches breast, nose does not
- Lips flanged
- Ear wiggles with suckle
- Soft cheeks
- Baby is relaxed



Signs of an Incorrect Latch

- Pain
- Clicking or sucking noises
- Cheeks sucked inward
- Baby easily coming off breast
- Little or no swallowing at breast
- Lips tucked in instead of flanged
- Mother's nipple flattened or creased after feeding
- Inadequate wet diapers

Natural Foods That Are Good for Milk Supply

- Fenugreek, Fennel, Moringa
- Oats with nuts and seeds
- Whole grains/Millets
- Flavouring agents like garlic/jeera
- Small variety fish

Expressed Breast Milk (EBM) or Formula Top-up

- EBM is the preferred top-up. Formula only with pediatrician's consent.
- Use paaladai, cup, or spoon for top-ups—do not pour milk directly into baby's mouth. Don't use feeding bottles. Sit baby up and let baby suck from paaladai.

Signs of Successful Breastfeeding

- Six to eight wet cloth diapers in 24 hours
- Two to five bowel movements in 24 hours
    - Remember, EBF babies may even go up to a week without a bowel movement
- Baby feeds every 2-3 hours
- Good weight gain

When to Seek Help

- Urine output is less than six times in 24 hours
- Baby looks dull or sleepy all the time, does not take feeds
- Nipple pain or cracking or severe engorgement
- Signs of jaundice (yellowing skin or eyes)

Follow-up Post Discharge

- Book a Review Appointment with Lactation Consultant
- Available on **Monday, Wednesday and Friday (11 AM - 2 PM)**

FAQs

- **Why am I feeling abdominal cramping and bleeding during breastfeeding?**
  Mild cramping and bleeding occur due to oxytocin release, which helps the uterus contract.
- **Is it important to wear a bra while breastfeeding?**
  Yes, a well-fitted nursing bra can provide comfort and support.
- **I am having pain while feeding and my nipples feel sore. Is it expected while feeding?**
  Painful and sore nipples indicate latch issues. Visit your Lactation consultant and get it corrected as soon as possible.
- **My nipple is cracked and bleeding. Can I feed the baby on the painful breast?**
  Yes, but get your latch corrected and consider using nipple cream or expressed milk for healing.
- **I am facing severe engorgement and pain. What do I do?**
  Frequent feeding, gentle massage, cold compresses, and hand expression can help relieve engorgement. For severe engorgement, you can use chilled cabbage leaves. You can also use warm water (Not HOT) with Epsom salt soak for relief
- **How long should I burp my baby after feeding?**
  Burp for about 5-10 minutes or until your baby releases trapped air. Hold your baby upright on your shoulder or sit them on your lap and pat on their back to burp. Remember, you may not hear a burping noise all times.

- **Can I breastfeed in the reclined position?**
  Yes, but make sure your baby's head is elevated and ensure nose and ears are not pressed.
- **I am feeding often, and my supply is good, but my baby is not gaining weight. Why?**
  Check if the baby is receiving hind milk. Talk to your Lactation Consultant about your position & latch.
- **Every time my baby cries, should I feed her/him?**
  Not necessarily—crying can indicate other needs like sleep, comfort, or a diaper change. Sometimes, babies just need to be held and soothed. Consider using white noise for comfort.
- **How can I improve my milk supply?**
  Frequent nursing, proper hydration, a balanced diet, and skin-to-skin contact can help. Ensure baby empties at least one breast during a feed to stimulate the demand-supply cycle. Talk to your Lactation Consultant about power pumping or use of galactagogues.
- **How will I know if my baby is getting enough milk?**
  Signs include steady weight gain, sufficient wet nappies, adequate sleep of 2-3 hours after every feed and a satisfied baby.

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